



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Guideline

Older adult graduate referral admission

Reference #: FSFHG-MENH-GUI-0014

Scope

Site	Service/Department/Unit	Disciplines
Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health and Administration

1. Introduction

This guideline provides a detailed account of the referral criteria and process for referring adult mental health consumers to the Fremantle Older Adult Mental Health Service (FOAMHS). All referrals are assessed and prioritised by the multidisciplinary team (MDT).

2. Terminology

Cambridge Cognition Examination	CAMCOG – standardised instrument used to measure the extent of dementia, and to assess the level of cognitive impairment.
Care Coordinator	Nominated person who has responsibility for coordinating, facilitating and integrating mental health treatment, care and support to a mental health consumer, their family/carers and significant others.
Older Adult	As per the Australian Institute of Health and Welfare, an older adult is defined as an individual aged 65 years and over, whilst for older Aboriginal and Torres Strait Islander (ATSI) people, the age range 50 years and over is used to reflect the gap in life expectancy between Aboriginal and non-Aboriginal Australians.

Graduate	A mental health consumer who: • Is an older adult • Has an established diagnosis that meets referral criteria • Has been accepted for transfer to FOAMHS
Montreal Cognitive Assessment	MoCA - widely used screening assessment tool for detecting cognitive impairment.
Rowland Universal Dementia Assessment Scale	RUDAS – short cognitive screening instrument designed to minimise the effects of cultural learning and language diversity on the assessment of baseline cognitive performance.

3. Guideline

3.1. Referral criteria

- Consumer referred to FOAMHS as a graduate from the adult service will have:
 - Been active and have received treatment from the adult mental health service for a primary mental health diagnosis within the last 2 years.
 - A comprehensive MDT review including psychiatric history, comprehensive risk assessment and management plan.
 - Eligibility for Aged Care Assessment Team (ACAT) assessment likely to identify significant or progressive age-related deficits
 - Additional screening/investigations to exclude reversible causes of any cognitive decline including pathology tests and brain computed tomography (CT) scan /magnetic resonance imaging (MRI).

3.1.1. Inclusion Criteria

- confirmed diagnosis of a neurodegenerative disease with accompanying cognitive decline.
- evidence of significant and irreversible cognitive decline related to an enduring mental illness and/or negative medication related side effects that significantly impact functional ability.
- evidence of behavioural and psychological symptoms of dementia (BPSD)
- current NDIS package.
- a formal cognitive assessment including but not limited to:
 - MoCA
 - RUDAS

- Addenbrooke's Cognitive Examination
- CAMCOG The potential for transfer of care to FOAMHS must have been discussed with the patient and carer.
- Residential Aged Care Facility as a permanent placement for minimum period of three months.

3.1.2. Exclusion criteria

- Evidence of significant and irreversible cognitive decline primarily related to:
 - alcohol/substance use,
 - head injury,
 - recent neurosurgery, brain tumour or meet the criteria for the State-wide Neurosciences Unit e.g. Huntington's disease

Care coordination remains with the referring adult treating team until a documented agreement (accepted at MDT) is reached. The consumers care will then transfer to FOAMHS.

3.2. Referral process

- Following adult MDT endorsement, the adult treating team care coordinator discusses referral with FOAMHS and submits a written referral via e-referral making explicit referral criteria, reason for transfer of care, and patient/appointed guardian/carers consent for transfer of care to OAMH. This referral may include Consultant to Consultant liaison.
- FOAMHS will confirm referral criteria are met, including potential benefits of transfer to OAMH prior to accepting referral.
- On acceptance, a transfer of care summary is completed to support clinical handover of patient care. The referring Care Coordinator may arrange a multidisciplinary team meeting to include:
 - Consumer
 - Carer/s (and support person if required)
 - Care Coordinator – adult
 - Consultant or Registrar – adult
 - Consultant or Registrar – FOAMHS
 - FOAMHS Triage Officer or assigned Care Coordinator
- If the referral is not accepted the referring team will be informed. The FOAMHS Triage Officer and/ or Consultant will discuss reasons referral declined with the referring team Care Coordinator and/ or Consultant. The adult treatment will remain with the adult treating team.

- Complex referrals will be escalated to the Head of Service for further review in consultation with the OAMH Program Manager

3.3. Referral process – inpatient outliers

- On occasion inpatient care may be provided outside established bed allocation. E.g. an adult receiving care within W4.3 or the OA HTT; or an older adult receiving care on an adult ward. Reasons may include:
 - Bed pressure – consumer remains with their originating service stream (adult or older adult)
 - Specific OA ward environment required – will not automatically trigger transfer of care to OAMH. Referral to be reviewed in line with Graduate Referral criteria 3.1.
 - Adult admission to OA HTT – on discharge, transfer of care to be considered in line with Graduate Referral criteria 3.1 as above.

4. Compliance/Performance Monitoring

FOAMHS will monitor compliance via comparison of graduate referrals received against referral criteria. Referrals which do not meet the referral criteria will be declined and discussed with referrers.

5. Related Policy Documents

- SMHS:
 - [SMHS Care coordination in mental health](#)
 - [Rights of carers and other personal support persons](#)
- FSFHG:
 - Clinical Documentation policy
 - Clinical Handover policy
 - Clinical risk assessment and management
 - Older Adult Mental Health referrals

6. Related Standards

- NSQHS Standards:
 - Clinical governance
 - Partnering with consumers
 - Communicating for safety

FSFHG-MENH-GUI-0014	Version 5.0	Endorsed March 2026	Review Due Feb 2029	Page 4 of 5
---------------------	-------------	---------------------	---------------------	-------------

The hard copy of this document may be out of date. To ensure you are reading the current version, check the policy and guideline site on the FSFHG Policy Intranet.

- Chief Psychiatrist Standards for Clinical Care:
 - Assessment
 - Transfer of Care

7. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	10/2015	Clinical Nurse, Mental Health		FH Clinical Governance Committee Mental Health Service	03/2020
2	06/2019	Program Manager, Older Adult	MH SQR Committee	FSFHG policy Committee	06/2022
3	03/2022	Program Manager, Older Adult	NMPGC	S 5 SQR	03/2026
4	02/2026	Program Manager, Older Adult	FOAMHS Management Group	FSFH Clinical Governance Sub Committee	02/2029
5	03/2026	Program Manager Older Adult	FOAMHS Management Group	FSFH Clinical Governance Sub Committee	02/2029